



POLICY	
Management of Clinical Alerts	
Scope (Staff):	All NMHS Mental Health Staff
Scope (Area):	All NMHS Mental Health Areas

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1. Aim

North Metropolitan Health Service Mental Health (NMHS MH) promotes the provision of efficient and appropriate emergency and ongoing care for patients and a safe environment for patients, carers and staff by ensuring clinical risks are appropriately assessed, recorded and managed as per the [MP 0053/17 WA Health Patient Alert Policy](#) , the [NMHS MH Clinical Risk Assessment and Management \(CRAM\) Policy](#) and the [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#).

To ensure compliance with these policies clinical services must have formal processes in place for ensuring risks are assessed, recorded appropriately and regularly reviewed for integrity and accuracy of information within the clinical record, PSOLIS and the relevant Patient Administration System (PAS).

2. Background

NMHS Mental Health services manage a complex set of clinical risks posed to consumers, staff and other members of the community that require communication to service providers, across the following categories:

- Behavioural (e.g. aggression, suicide)
- Medical (e.g. allergies, or any medical condition including hearing, swallowing, falls, pressure injury, speech and/or sight impairments)
- Micro-biological
- Social
- Medication (i.e. sensitivities, allergies or adverse reaction)
- Other (e.g. other alerts such as a history of intravenous drug abuse)

Please refer to:

- [MP0053/17 Patient Alert Procedure to Adverse Drug Reactions](#)
- [MP0053/17 Patient Alert Procedure for Dietary/Food Allergens](#)
- [MP 0053/17 Guidelines for Medical Conditional and Medication-related Patient Alerts](#)

Life Threatening Alerts relevant to Emergency Departments

The [MP 0053/17 WA Health Patient Alert Policy](#) was introduced to clarify the clinical alerts required to support Emergency Department personnel in situations where patients were unable to communicate their life-threatening risks at presentation. These alerts, critical for emergency clinicians' awareness prior to commencement of medical treatment, relate to specific anaesthetic, drug reactions and medical conditions and must be recorded on the PAS (currently WebPAS).

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☒

4. Definitions

Allergy: An abnormal immunologic response following exposure to an allergen.

Anaphylaxis: An acute, potentially life-threatening syndrome caused by sudden release of mast cell mediators in reaction to exposure to an allergen.

Allergies (including anaphylactic reactions) require a Clinical Alert on PAS.

ADVERSE DRUG REACTION (ADR): A harmful or undesirable effect associated with the exposure to a medication at therapeutic or sub-therapeutic doses. Adverse drug reactions may or may not be escalated to serious [adverse] drug reactions requiring a Clinical Alert. A serious adverse drug reaction which requires a Clinical Alert on PAS is defined as a reaction that may lead to a life-threatening event and has an absolute or relative contraindication to repeat administration of the drug. (Ref: [MP 0053/17 WA Health Patient Alert Policy](#))

CLINICAL ALERT: There are 2 types of clinical alerts that highlight risks relevant to Mental Health patients:

- **In an Emergency Hospital Setting:** A diagnosis which has the potential to be of critical importance to patients' management during the first 24 hours of their admission to hospital (general) and assumes that the patient is not always capable of communicating such information. The Clinical Alerts (MedAlerts, Micro Alerts and Risk Alerts) function within the PAS is accessible for viewing across all public hospitals in Western Australia. Refer to the [MP 0053/17 WA Health Patient Alert Policy](#). WebPAS is the current PAS used across WA Health.
- **In a Mental Health Setting:** Alerts provide immediate information regarding risk factors that increase the vulnerability of the patients, staff, relatives or other patients to physical, environmental, social or psychological harm that should be taken into consideration when assessing and treating the consumer. (Ref: [NMHS MH Clinical Risk Assessment and Management \(CRAM\) Policy](#))

DIGITAL MEDICAL RECORD (DMR) – is a digitalised medical record containing auto published documents from integrated application, eforms and scanned documents and superseded the hardcopy medical record at many health services.

WORK IN PROGRESS FILE (WIP) (DMR SITES ONLY): a temporary paper-based healthcare record that contains the paper forms used for a patient during an episode of care, prior to the forms being scanned into the digital medical record.

ICM - iSoft Clinical Manager, the system that contains pathology and other diagnostic results for patients. This system receives a data feed of patient alerts from WebPAS.

PAS: webPAS - The Web based Patient Administration System.

PSOLIS/webPSOLIS: The Psychiatric On Line Information System. WebPSOLIS sits in parallel to the existing PSOLIS Java and receives a data feed of patient alerts from WebPAS.

RISK: The chance of an event occurring that will have an impact upon safety, values, goals or intentions. It is assessed in terms of repercussions and likelihood (Ref: [NMHS MH Clinical Risk Assessment and Management \(CRAM\) Policy](#)).

DISASTER FLAG: A flag that can be switched on in the PAS to notify clinicians of a microbiological risk related to a pandemic, epidemic, external emergency or natural disaster. The disaster code that specifically relates to COVID-19 alerts clinicians that a patient is either a suspected or confirmed covid-19 case awaiting further pathology results or medical clearance.

5. Procedures

On entry to service the triage officer (Community Services) or admitting clinician (Inpatient Services) is responsible for updating and printing PSOLIS alerts and the medical record clerk/admitting clerk is responsible for updating the WebPAS alerts as instructed by the



admitting clinicians. The PSOLIS and WebPAS Alert lists should be printed from WebPSOLIS using the 'Print All' function and filed in the front of the patient's Work In Progress File (DMR sites only). Non-DMR sites should file the Alerts lists behind the red risk divider in the patient's medical record. An Alerts sticker should be adhered to the front of the Work in Progress File (DMR sites) or the medical record (non-DMR sites).

5.1 Documenting and Communicating Clinical Alerts

For mental health clinicians PSOLIS is the main repository for adding/ reviewing/ removing/ updating clinical alerts. All current PSOLIS and WebPAS Alerts can be viewed in WebPSOLIS. Clinicians can also view pre-existing clinical alerts on the front page of the DMR clinical record. The alerts displayed are only from WebPas, NaCS and other hospital systems such as ADA (Allergy and Diet Application – not in use at MHS). Also an Alert Flag displays on the DMR Precis Page to alert users of any current active alerts from these systems. See **Appendix 2** for DMR screenshot examples.

5.1.1 Assessment of Risks/Alerts:

Refer to table 1 in section 5.1.3 for Clinical Alert Categories and Recording Requirements

Risks and Alerts need to be assessed at:

- entry to the service,
- when there is a significant change in the consumer's presentation or circumstances, (either deterioration or improvement)
- When there is a change to medications
- at formal Multidisciplinary Team (MDT) clinical review of the management plan at a minimum of three monthly, and
- at discharge or on transfer of care.

Identify any previous adverse reactions to medication/allergies and or/diet related and any existing anaesthetic or medical conditions by interviewing the patient, their carer or referencing the patient's medical record/WebPAS / PSOLIS and My Health Record where possible. Assess the previous reactions and conditions to determine whether they are known and documented Alerts/Risks or if they classify as a new patient alert/risk. For new Risks/Alerts follow the process outlined in 5.1.2.

To identify any risks, the relevant Statewide Standardised Clinical Documentation (SSCD) form(s) as per WA Health Operational Directive 0526/14 and any other endorsed risk assessment tools and documents are to be used (e.g. the Progressive Risk Assessment , Risk Assessment and Management Plan (RAMP), Child and Adolescent RAMP (CRAMP), Mental Health Assessment, falls risk assessment tool, etc). For new Risks/Alerts follow the process outlined in the 5.1.2.

If risks/alerts are already known and documented ensure:

- Alerts sticker is on the front of the hard copy medical record (non DMR sites)/ Work in Progress file (DMR sites)
- For Adverse Drug Reaction (ADR) Allergies ensure the ADR is documented on the WA Hospital Medication Chart (WA HMC) and an ADR sticker is applied to the WA Hospital Medication Chart.
- For Adverse Drug Reactions/Allergies ensure a Red ID band is placed on the patient's wrist (if ID bands utilised for patient group).

-
- PSOLIS and WebPAS alert lists are printed and filed at the front of the Medical Record (Non-DMR site), or WIP file (DMR site).

Business Continuity Plans for PSOLIS, WebPAS and the DMR should be utilised where system downtime occurs, to mitigate the risk of not being able to access clinical alert information.

5.1.2 New Risks/Alerts Identified:

Where a new risk/alert is identified the treating consultant for the patient will ensure that the following takes place and adequate risk management plan is developed:

- Where a service is using a hardcopy medical record instead of the Digital Medical Record (DMR) an ALERT sticker is placed on the front cover of the current volume of the patient's medical record. For DMR sites, the Alert sticker is to be placed on the front cover of the Work in Progress file. See **Appendix 1** for a list of available alert stickers when using a hardcopy medical record which should be adhered to.
- The risk will be recorded on PSOLIS. Refer to the [Psychiatric Services Online Information System \(PSOLIS\) Alerts Process Guide Version 0.5](#) for further information. A copy of the PSOLIS Alerts List should be printed each time a new Alert is added or modified in PSOLIS and this document should be added to the hardcopy medical record or, where the DMR is in use, to the patient's work in progress file.
- For any new patient alert that is Anaesthetic/Medical Condition/ Medication/allergy or dietary related risk following process is to be followed:
 - The clinician who has identified the new alert should send an email to email the treating Consultant, for validation. the Treating Consultant who validates the alert and sends an email to the relevant site's Health Information Manager, Area System Administrator or Medical Record Supervisor detailing alert information that is required to be recorded on the PAS and PSOLIS, where possible, within 24 hours of receipt. The Head of Medical Services must be copied into the email.
 - A copy of the email is sent to the NMHS MH Medication Safety and Audit Pharmacist, if the risk meets the medication 'drug reaction' category. Thereafter, the Chief Pharmacist or Head of Medical Services will approve the notification in accordance with this policy and immediately email the Area Health Information Manager, Medical Record Supervisor or Area System Administrator the relevant alert information to be recorded on the PAS. The Head of Medical Services and Treating Consultant should be copied into the email.

NB: Anaesthetic/Medical Condition Alerts recorded on PAS are limited to those detailed in the [Patient Alert Procedure for Adverse Drug Reactions](#) and the [MP 0053/17 WA Health Patient Alert Policy](#). Additional conditions not listed within the state-wide policy must be submitted for approval via the relevant site's Health Information Manager, to the WA Clinical Alert Business Group.

5.1.3 Table 1: Clinical Alert Categories and Recording Requirements

Type of Clinical Alert:	Recorded on PSOLIS and copy of updated PSOLIS Alert List printed for DMR or hardcopy medical record	Record on PAS (see procedure for coders)
Behavioural	✓	
Medical	✓	*Anaesthetic *Medical conditions (Refer to MP 0053/17 WA DoH Patient Alert Policy and Patient Alert Procedure for Adverse Drug Reactions)
Micro-biological	✓	✓
Social	✓	
Medication	✓	(Refer to MP 0053/17 WA DoH Patient Alert Policy and Patient Alert Procedure for Adverse Drug Reactions)
Other	✓	

5.1.4 Allergies and Adverse Drug Reactions

Where an allergy or Adverse Drug Reactions (ADR) is identified, the treating Psychiatrist will need to ensure that the following occurs:

- Medical staff/ Pharmacist to complete the WA Hospital Medication Chart ADR section and an ADR sticker must be applied to the chart.
- An RED ID band is placed on the patient's wrist (if ID bands utilised for patient group).
- The allergy/ ADR details are included on PSOLIS.
- Providing and documenting the information given to the patient about the ADR, and their carer/next of kin/nominated other as required.

5.1.5 Advance Health Directives

When a patient presents to a health service it is important to identify if the patient has an Advance Health Directive (AHD) to assist with advance care planning. Where a patient has an AHD, a Medical Alert should be added to the PAS using an alert code of Advance Health Directive. The Alert should also be added in PSOLIS. Where the patient has a copy of the AHD an additional copy should be made and filed behind the Risk Divider in the current volume of the patient's medical record (for non DMR sites). For DMR sites the AHD will be filed in the front of the patients work in progress file. At all sites it should be filed after the PMR00000 Voluntary Assisted Dying First Request Form.

For DMR sites, please note, AHD that have been already scanned into DMR can be found in the Summary folder of the DMR clinical Record.

For further information on recording AHDs as an Alert in the PAS refer to the [MP 0053/17 WA Health Patient Alert Policy](#) Microbiological (Micro) Risks

5.1.6 Microbiological Alerts

Where a new microbiological (Micro) risk is identified, the treating Psychiatrist will need to ensure the following occurs:

- a) Compliance with the [NMHS MHPHDS IPC: Multi-Resistant Organisms Infection Control Guidelines](#);
- b) PSOLIS is updated;
- c) The NMHS MH Infection Prevention and Control (IPC) Department is informed.

NMHS MH IPC will confirm that a positive pathology result has been received for a multi-resistant organism. If certain screening criteria are met IPC will add the Micro Alert to WebPAS, inform the clinical team to document the micro-alert in the patient's medical record and on PSOLIS.

Where a patient is admitted with a known Micro Alert (i.e. already registered on webPAS) the service must have systems in place, e.g. via clerical or triage personnel, to advise the treating clinician and ensure IPC has reviewed the alert as required.

NMHS MH IPC will notify the WebPAS data entry staff at the relevant site to activate the COVID-19 disaster flag for a patient in WebPAS when certain clinical criteria are met. This is for the purposes of flagging the patient's clinical risk of covid-19.

5.1.7 Removal of Redundant Risks/Alerts

Where a risk/alert listed in the clinical record, PAS or PSOLIS are no longer relevant, the treating Psychiatrist (in consultation with HOCS or Case Manager as required) must ensure that the medical record is updated to accurately reflect current risks.

Where cessation of a Risk/ Alert is approved:

- The Treating Consultant will notify the Health Information Manager, Medical Record Supervisor or Area System Administrator via email who will arrange removal from PAS and PSOLIS. The Head of Clinical Services should be copied into the email. An explanation and rationale for cessation of risk must be documented in the patient notes, either on the DMR or for sites not using the DMR, in the integrated progress notes.
- **NB Microbiological alerts must only be inactivated by NMHS MH IPC staff.** NMHS MH IPC will request that the treating team conducts pathology testing for clearance and they will inactivate a microbiological alert in WebPAS when a negative result is received for the patient. IPC will notify the ward CNS once inactivation of the alert has been carried out in WebPAS

Where a patient has a COVID-19 disaster flag active on their WebPAS record and meets certain clinical criteria, NMHS MH IPC staff will notify staff within the WebPAS office at the relevant site to remove the WebPAS disaster flag.

6. Compliance and Evaluation

A sample of patients will be audited annually to ensure compliance with this policy

100% of audited patients with med alerts and micro alerts will be recorded on PAS and PSOLIS



100% of eligible medication alerts recorded on the WA Hospital Medication Charts of audited patients will have a corresponding alert on PSOLIS

Related internal policies, procedures and guidelines
DoH MP 0053/17 WA DoH Health Patient Alert Policy MP0053/17 Patient Alert Procedure to Adverse Drug Reactions MP0053/17 Patient Alert Procedure for Dietary/Food Allergens WA DoH Adverse Drug Reaction Information Brochure for Consumers MP 0078/18 Medication Chart Policy MH NMHS MHPHDS IPC: Multi-Resistant Organisms Infection Control Guidelines NMHS MH Clinical Record Documentation Procedure NMHS MH Clinical Risk Assessment and Management (CRAM) Policy COVID-19 Related Information HSS PSOLIS Alerts Process v 0.5



Policy Sponsor	NMHS MH Director of Medical Services					
Policy Contact	NMHS MHPHDS Manager, Area Health Information					
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	NMHS Mental Health Clinical and Corporate Information Management Committee			Date:	16/10/2024	
Endorsed by:	NMHS MH Director of Medical Services			Date:	23/10/2024	
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☒  Std 4: Medication Safety			☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care					
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Document Version Control			
Date	Version	Author	Notes
21/8/2015	V1.0	Director of Safety Quality and Performance NMHS MH	Initial endorsement
12/04/2022	V2.0	A/Policy and Audit Officer	Updated process to include new SSCD forms and new processes for adding alerts to WebPAS and PSOLIS. Updated section on Micro Alerts to reflect new process detailed by NMHS MH Infection Prevention and Control re adding and removal of Micro Alerts from WebPAS. Updated links to reflect current versions of related policies and business rules. Added covid-19 related protocols.

01/11/2022	V2.1	Policy and Audit Officer	Minor amendment to Policy re red risk divider
23/10/2024	V3.0	Manager, Area Health Information	Updates made to remove the use of the PMR01 in view of implementation of the Digital Medical Record (DMR) at most health services within NMHS Mental Health. Minor corrections made to section on adverse/severe drug reactions.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD number: 1324)

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Appendix 1: Alert Stickers (for non DMR sites/services)

DYSPHAGIA ALERT

This patient has an identified
risk of choking.

Please see speech pathology report
before offering food or drink.

**DRUG
ALERT**

ADVERSE DRUG REACTION

Appendix 2: Screenshot of the DMR Clinical Record Front Page showing active and revoked alerts from WebPAS, NACS, ADA (food allergy system is not used at Graylands Hospital).



Government of **Western Australia**
Department of **Health**

OPAL, TEST PATIENT 11
URN: B1122551

CONFIDENTIAL MEDICAL RECORD
[Patient Confidentiality - Information Access, Use & Disclosure Policy](#)

CLINICAL RECORD

INPATIENTS

COMMUNITY

OUTPATIENTS

ADMISSIONS

CONSENT

LEGAL

INVESTIGATIONS

DIAGNOSTICS

IMMUNISATION

Active Alerts (3)

Criticality

Verified: Source System

Patient Risk Log

Verified: Source System

NMRSA POSITIVE (NON-MULTIRESISTANT)

Verified: Source System

P Other

Revoked Alerts (4)

Previous

Next

Add To Consults

Clinical Record

Results Review

Star

Patient Risk Log
Patient has 2 other alerts

Favourites

Search

Clinical Letter

Progress Note - Medical

Progress Note - Team Conference

Progress Note - Telephone Adhoc FSH

Inpatient Attendance - Cardiology (DR MARINA) Current

Name: TEST PATIENT 11 OPAL (B1122551)

Patient DOB: 27 Jun 2024

Admission Date: 16 Jul 2024 12:58

Diagnosis: TESTING

Insurance: Public

Ward: 3B

Discharge Date: NA

Précis : Medical

OPAL, TEST PATIENT 11 (MSTR)

Born: 27 Jun 2024 (6 weeks)

Gender: Male

B1122551

Location: 3B

Admission: Jul 16 2024 12:58PM

Inpatients

Financial election: Public

LOS (days): 28